

# Loss of Appetite (Anorexia) and Cachexia

Cancer and cancer treatment can increase how much energy your body needs. At the same time, you might lose your appetite (anorexia). This can lead to cachexia.

- Anorexia is the loss of appetite. If you lose your appetite, you are likely to eat less. This can cause you to lose weight and make you feel more tired and weak.
- Cachexia develops when people with cancer totally lose their appetite. This leads to both weight and muscle loss. Cachexia is most common in people with [advanced cancer](#)<sup>1</sup>.

Both anorexia and cachexia can affect your quality of life and ability to do usual activities. They can also affect how well you can handle your cancer treatment.

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## Anorexia

**What causes loss of appetite (anorexia)?**

People with cancer lose their appetite for many reasons.

Tumors that are in or around parts of the digestive tract might make it harder to eat.

They might cause trouble swallowing or make you feel full after a small amount.

- Hormones released by some cancers so the body doesn't know that it's hungry.
- [Changes in taste or smell](#)<sup>2</sup>
- [Pain](#)<sup>3</sup>
- [Nausea or vomiting](#)<sup>4</sup>
- [Constipation](#)<sup>5</sup> or [diarrhea](#)<sup>6</sup>
- Stress, [depression](#)<sup>7</sup>, and [dehydration](#)<sup>8</sup>, can reduce a person's appetite.
- Other health problems and medicines used to treat them can also cause loss of appetite.

### **Symptoms of loss of appetite**

- Eating less than normal or not eating at all.
- Little or no interest in food
- Turning down favorite foods
- Weight loss

Loss of appetite is common with some cancers and treatments. It can lead to weight loss, weakness, and other problems. These problems can make it harder for you to cope with your treatment.

Sometimes, people lose their appetite for a short while, but some people lose their appetite for days or weeks. Let your cancer care team know if your appetite changes as soon as you notice it, before, during, or after treatment. Letting them know about appetite changes can help limit problems from losing weight and not getting enough food and fluids.

### **Treatment for loss of appetite**

There are things the cancer care team can do to help if you have lost your appetite. They might be able to:

- Treat symptoms that are making it harder to eat

- Have you work with a registered dietitian nutritionist (RDN) or registered dietitian (RD). They can give you ideas about how to get more food and fluids.
- Suggest medicines that might help increase appetite
  - Prescription medicines such as metoclopramide, megestrol acetate, olanzapine, and dexamethasone (steroid)
  - Over-the-counter medicines for gas and other problems that may not need a prescription
  - [Cannabis \(marijuana\)](#)<sup>9</sup> is being studied for its [possible benefits](#)<sup>10</sup> in helping to manage loss of appetite and nausea from cancer and its treatment. But cannabis has [risks](#)<sup>11</sup> and can be harmful in certain situations, forms, and doses. Two [cannabinoid medicines](#)<sup>12</sup> have FDA approval to help manage nausea: dronabinol and nabilone. Both require a prescription.

### Tips to manage loss of appetite

Eating as well as you can is an important part of dealing with cancer, treatments, and side effects. Here are some hints that may help if your appetite has changed:

- Eat when you feel hungry. You don't have to wait for set mealtimes.
- Eat several small meals or snacks throughout the day, rather than 3 larger meals.
- Eat your favorite foods any time of the day; for instance, if you like breakfast foods, you can eat them for dinner.
- Try taking only small sips of liquids while eating. This can help keep you from feeling full early.
- Try to drink most of your liquids between meals.
- If the smell of food bothers you, try eating foods that are cold or at room temperature.
- Make eating more enjoyable by eating with your family or friends, choosing your favorite foods, setting the table with pretty dishes, playing your favorite music, or watching TV.
- Be as active as you can. Start slowly and increase your activity over time.
- If you have nausea or pain, take medicines given to you by your cancer care team about 30 minutes before you plan to eat.
- Keep a variety of protein-rich snacks on hand that are easy to prepare and eat. These include yogurt, cereal and milk, half a sandwich, a bowl of hearty soup, and cheese and crackers.

- Review the [tips on getting more calories and protein](#)<sup>13</sup> and try to include these in meals and snacks throughout the day.

## Talking with your cancer care team about anorexia

Let your cancer care team know if you have any of the following symptoms:

- Feel sick to your stomach and can't eat at all for a day or more
- Vomit for more than 24 hours
- Are unable to drink or keep down liquids
- Lose 3 pounds or more in a week (or less than a week)
- Don't urinate (pee) often, and when you do, it comes out in small amounts, smells strong, or is dark colored
- Don't urinate (pee) for an entire day
- Don't move your bowels (poop) for 2 days or more

## Cancer cachexia

### What causes cancer cachexia?

Cachexia develops when:

- The body needs more energy (calories) to handle changes caused by cancer or cancer treatments.
- The person with cancer loses their appetite and can't eat enough calories to meet the body's need.

The person with cancer cachexia loses muscle as well as fat and overall weight. This can lead to weakness and fatigue, and possibly not being able to do usual daily activities.

A person with cachexia may look very thin. If they had excess weight before they had cancer, they may just look as if they've lost weight. Sometimes blood tests are needed to find out if someone has cachexia.

Cancer cachexia can cause low levels of some nutrients, such as protein and certain vitamins and minerals, and can be life-threatening. People with cancer who lose weight without trying should let their cancer care team know.

## Treatment of cachexia

People with cachexia need to get more calories and protein to offset the loss of weight, muscle, and other nutrients. Steps to treat this may include:

- Working with a registered dietitian nutritionist (RDN) or registered dietitian (RD) to come up with a plan to stop losing weight. They will most often suggest a high-protein, high-calorie diet with nutrient-rich foods. This may include different supplements or ways of preparing food. They can also offer tips on how to deal with loss of appetite.
- There are some medicines that the cancer care team can prescribe that might help increase appetite or help with other symptoms that may lower appetite.
- Trying tips to manage loss of appetite may be helpful as well.

## Tips for family, friends, and other caregivers

Appetite loss and cachexia can cause many symptoms that make eating and drinking very hard. And it can be hard when your loved one doesn't want to eat. **But know that whether they eat or not is out of your control.** Try not to make this a source of conflict.

- Know that your loved one can't control their loss of appetite. And you can't control whether they eat or not.
- Pushing your loved one to eat can **make the issue worse**. Try not to fight with them about eating.
- Be supportive when your loved one feels like they can eat. Sometimes having someone to eat with can make the meal more pleasant.
- Know that your loved one might ask for food, but by the time it's ready, they might not want it anymore. This is frustrating, but very common.

- Find ways to show you care that don't involve food. Offer to read to them, talk to them about things they enjoy, play a game, or watch a movie or TV together.
- If you are the primary caregiver, ask to talk with a dietitian for suggestions about how you can help your loved one.

## Talking with your cancer care team about cachexia

Let your cancer care team know if you have any of the following symptoms:

- Feel sick to your stomach and can't eat at all for a day or more
- Vomit for more than 24 hours
- Are unable to drink or keep down liquids
- Lose 3 pounds or more in a week (or less than a week)
- Don't urinate often, and when you do, it comes out in small amounts, smells strong, or has a dark color
- Don't urinate (pee) for an entire day
- Don't move your bowels (poop) for 2 days or more

## Hyperlinks

1. [www.cancer.org/cancer/supportive-care/advanced-cancer.html](http://www.cancer.org/cancer/supportive-care/advanced-cancer.html)
2. [www.cancer.org/cancer/managing-cancer/side-effects/eating-problems/taste-smell-changes.html](http://www.cancer.org/cancer/managing-cancer/side-effects/eating-problems/taste-smell-changes.html)
3. [www.cancer.org/cancer/managing-cancer/side-effects/pain.html](http://www.cancer.org/cancer/managing-cancer/side-effects/pain.html)
4. [www.cancer.org/cancer/managing-cancer/side-effects/eating-problems/nausea-and-vomiting.html](http://www.cancer.org/cancer/managing-cancer/side-effects/eating-problems/nausea-and-vomiting.html)
5. [www.cancer.org/cancer/managing-cancer/side-effects/stool-or-urine-changes/constipation.html](http://www.cancer.org/cancer/managing-cancer/side-effects/stool-or-urine-changes/constipation.html)
6. [www.cancer.org/cancer/managing-cancer/side-effects/stool-or-urine-changes/diarrhea.html](http://www.cancer.org/cancer/managing-cancer/side-effects/stool-or-urine-changes/diarrhea.html)
7. [www.cancer.org/cancer/managing-cancer/side-effects/emotional-mood-changes/depression.html](http://www.cancer.org/cancer/managing-cancer/side-effects/emotional-mood-changes/depression.html)
8. [www.cancer.org/cancer/managing-cancer/side-effects/eating-problems/fluids-and-dehydration.html](http://www.cancer.org/cancer/managing-cancer/side-effects/eating-problems/fluids-and-dehydration.html)
9. [www.cancer.org/cancer/supportive-care/integrative-medicine/cannabis.html](http://www.cancer.org/cancer/supportive-care/integrative-medicine/cannabis.html)

10. [www.cancer.org/cancer/supportive-care/integrative-medicine/cannabis/benefits-of-cannabis.html](https://www.cancer.org/cancer/supportive-care/integrative-medicine/cannabis/benefits-of-cannabis.html)
11. [www.cancer.org/cancer/supportive-care/integrative-medicine/cannabis/cannabis-risks.html](https://www.cancer.org/cancer/supportive-care/integrative-medicine/cannabis/cannabis-risks.html)
12. [www.cancer.org/cancer/supportive-care/integrative-medicine/cannabis.html](https://www.cancer.org/cancer/supportive-care/integrative-medicine/cannabis.html)
13. [www.cancer.org/cancer/survivorship/coping/nutrition/once-treatment-starts.html](https://www.cancer.org/cancer/survivorship/coping/nutrition/once-treatment-starts.html)
14. [www.cancer.org/cancer/survivorship/coping/nutrition/benefits.html](https://www.cancer.org/cancer/survivorship/coping/nutrition/benefits.html)

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